



Lomide®

PRODUCT NAME

Lomide Capsule 2 mg

NAME AND STRENGTH OF ACTIVE SUBSTANCE

Each capsule contains 2 mg Loperamide HCl

PRODUCT DESCRIPTION

White granule in light green-dark green capsule shell No.4 with "Siam" on both of capsule shell

PHARMACODYNAMICS

Pharmacotherapeutic group: Antipropulsives

Loperamide binds to the opiate receptor in the gut wall. Consequently, it inhibits the release of acetylcholine and prostaglandins, thereby reducing propulsive peristalsis, and increasing intestinal transit time. Loperamide increases the tone of the anal sphincter, thereby reducing incontinence and urgency.

PHARMACOKINETICS

Absorption

Most ingested loperamide is absorbed from the gut, but as a result of significant first pass metabolism, systemic bioavailability is only approximately 0.3%. Loperamide HCl formulations (hard and soft capsules, coated and uncoated tablets, chewable and orodispersible table, oral solution) are bioequivalent in terms of rate and extent of loperamide absorption.

Distribution

The plasma protein binding of loperamide is 95%, mainly to albumin.

Metabolism

Loperamide is almost completely extracted by the liver, where it is predominantly metabolized, conjugated, and excreted via the bile. Oxidative N-demethylation is the main metabolic pathway for loperamide, and is mediated through CYP3A4 and CYP2C8. Due to this remarkably high first pass effect, plasma concentrations of unchanged drug remain extremely low.

Excretion

The half-life of loperamide is about 11 hours with a range of 9-14 hours. Excretion of the unchanged loperamide and the metabolites mainly occurs through the faeces.

Pediatric population

No pharmacokinetic studies were performed in pediatric population. It is expected that pharmacokinetic behavior of loperamide will be similar to those in adults.

INDICATION

Use for the control and symptomatic relief of acute and chronic diarrhea. In patients with an ileostomy it can be used to reduce the number and volume of stools and to harden their consistency.

DOSAGE AND ADMINISTRATION

Adults and Paediatrics 6 to 17 years

Capsules: The capsules should be taken with liquid

Acute diarrhoea: The initial dose is 2 capsules (4 mg) for adults and 1 capsule (2 mg) for children; followed by 1 capsule (2 mg) after every subsequent loose stool.

Chronic diarrhoea: The initial dose is 2 capsules (4 mg) daily for adults and 1 capsule (2 mg) daily for children; this initial dose should be adjusted until 1-2 solid stools a day are obtained, which is usually achieved with a maintenance dose of 1-6 capsules (2-12 mg) daily.

The maximum dose for acute and chronic diarrhoea is 8 capsules (16 mg) daily for adults; in children it must be related to the body weight (3 capsules/20 kg) but should not exceed maximum of 8 capsules per day.

Elderly

No dose adjustment is required for the elderly.

Renal impairment

No dose adjustment is required for patients with renal impairment.

Hepatic impairment

Although no pharmacokinetic data are available in patients with hepatic impairment, Loperamide HCl should be used with caution in such patients because of reduced first pass metabolism (see *Warnings and Precautions*).

CONTRAINDICATIONS

- Loperamide is contraindicated in patients with a known hypersensitivity to loperamide or to any of the excipients
- Loperamide should not be used in children under 6 years of age
- Loperamide should not be used as the primary therapy in:
 - Patients with acute dysentery, which is characterized by blood in stools and high fever
 - Patients with acute ulcerative colitis
 - Patients with bacterial enterocolitis caused by invasive organisms including Salmonella, Shigella, and Campylobacter
 - Patients with pseudomembranous colitis associated with the use of broad-spectrum antibiotics
- Loperamide should not be used when inhibition of peristalsis is to be avoided due to the possible risk of significant sequelae including ileus, megacolon, and toxic megacolon. Loperamide must be discontinued promptly when constipation, abdominal distension or ileus develop

WARNINGS AND PRECAUTIONS

Loperamide is not recommended for children under 6 years of age. Its use has been associated with fatal episodes of paralytic ileus in infants and young children.

Appropriate fluid and electrolyte therapy should be given to protect against dehydration in all cases of diarrhoea. Oral rehydration therapy which is the use of appropriate fluids including oral rehydration salts remains the most effective treatment for dehydration due to diarrhoea. The intake of as much of these fluids as possible is therefore imperative. Drug-induced inhibition of peristalsis may result in fluid retention in the intestine, which may aggravate and mask dehydration and depletion of electrolytes. If severe dehydration or electrolyte imbalance is present, loperamide should be withheld until appropriate corrective therapy has been initiated.

The use of higher than the recommended doses for control of the diarrhoea may cause abnormal heart rhythms and serious cardiac events leading to death. Cases of syncope and ventricular tachycardia have been reported in adult patients receiving the recommended dosage of loperamide. Some of these patients were taking other drugs or had other risk factors that may have increased their risk of cardiac adverse reactions.

Although no pharmacokinetic data are available in patients with hepatic impairment, Loperamide HCl should be used with caution in such patients because of reduced first pass metabolism. This medicine

Actual Size 100 %

DRAWING NO. LOM-2-02-L-ENG-MY-A0004

SENT DATE 19/04/22

must be used with caution in patients with hepatic impairment as it may result in a relative overdose leading to CNS toxicity.

Abuse and misuse of loperamide, as an opioid substitute, have been described in individuals with opioid addiction (See *Overdose and Treatment*).

INTERACTIONS WITH OTHER MEDICATIONS

Quinidine or Ritonavir

Non-clinical data shown loperamide is a P-glycoprotein substrate. Concomitant administration of loperamide with quinidine or ritonavir (both are P-glycoprotein inhibitors), resulted in 2-3 fold increase in loperamide plasma levels.

Itraconazole

Concomitant administration of loperamide and itraconazole (inhibitor of CYP3A4 and P-glycoprotein), resulted in 3-4 fold increase in loperamide plasma concentrations.

Ketoconazole

Concomitant administration of loperamide and itraconazole (inhibitor of CYP3A4 and P-glycoprotein), resulted in a 5 fold increase in loperamide plasma concentrations.

Desmopressin

Concomitant treatment with oral desmopressin resulted in a 3 fold increase of desmopressin plasma concentrations, presumably due to slower gastrointestinal motility.

PREGNANCY AND LACTATION

Pregnancy

Although there are no indications that loperamide possesses teratogenic or embryotoxic properties, the anticipated therapeutic benefits should be weighed against potential hazards before loperamide is given during pregnancy, especially during the first trimester.

Lactation

Small amount of loperamide may appear in human breast milk. Therefore, loperamide is not recommended during breast-feeding.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Tiredness, dizziness, or drowsiness may occur in the setting of diarrheal syndromes treated with loperamide. Therefore, it is advisable to use caution when driving a car or operating machinery.

UNDESIRABLE EFFECTS

The following adverse reactions have been reported in patients receiving loperamide and are classified according to system organ.

System Organ	Adverse Reactions
Nervous system disorders	: Dizziness, headache, somnolence
Eye disorders	: Miosis
Gastrointestinal disorders	: Constipation, flatulence, nausea, vomiting, dry mouth, abdominal pain, abdominal distension
Skin and subcutaneous tissue disorders	: Rash, angioedema, pruritus, urticaria
Renal and urinary disorders	: Urinary retention
General disorders	: Fatigue

Cardiac disorders

Post-marketing experience

: QT/QTc interval prolongation, Torsades de Pointes, other ventricular arrhythmias, cardiac arrest, syncope, and death (See *Warnings and Precautions*)

OVERDOSE AND TREATMENT

Overdose

In case of overdose (including relative overdose due to hepatic dysfunction), central nervous system (CNS) depression (stupor, coordination abnormality, somnolence, miosis, muscular hypertonia, and respiratory depression), urinary retention and ileus may occur. Children may be more sensitive to CNS effects than adults.

In individuals who have intentionally ingested overdoses (reported in doses from 40 mg up to 792 mg per day) of loperamide, prolongation of the QT/QTc interval, Torsades de Pointes, other ventricular arrhythmias, and cardiac arrest, have been observed (See *Warnings and Precautions*). Fatal cases have also been reported.

Abuse, misuse and/or overdose with excessively large doses of loperamide, may unmask Brugada syndrome.

Treatment

In case of overdose, ECG monitoring for QT interval prolongation should be initiated. If CNS symptoms of overdose occur, naloxone can be given as an antidote. Since the duration of action of loperamide is longer than that of naloxone (1-3 hours), repeated treatment with naloxone might be indicated. Therefore, the patient should be monitored closely for at least 48 hours in order to detect possible CNS depression.

STORAGE CONDITIONS

Store below 30°C

SHELF LIFE

3 years

DOSAGE FORMS AND PACKAGING AVAILABLE

Blister strips with 10 x 10 capsules (One box contains 100 capsules)

DATE OF REVISION OF PI

Date of revision: 25/03/2022

Manufacturer:-

SIAM BHEASACH CO., LTD.

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Product Registration Holder, Importer:-

SPG PHARMA (MALAYSIA) SDN. BHD.

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